AO-01 Childbirth / Labor

CLAIBORNE COUNTY EMS | MEDICAL-DIRECTOR APPROVED | SOURCE PDF | October 16, 2026

Scope: Use for labor, delivery, delivery complications, immediately postpartum care, and maternal/newborn coordination. Transport is preferred unless delivery is imminent or transport is unsafe.

Labor, delivery, and immediate postpartum pathway

1. Assess	Due date and gestation, gravida/para, contraction pattern, membrane rupture, bleeding, fetal movement, prior rapid delivery, and maternal symptoms.
2. Maternal emergency?	Severe bleeding, seizure, hypertension/hypotension, altered mental status, severe pain, or shock: begin supportive care and open AO-03 as indicated.
3. Inspect perineum	No digital vaginal examination. Look for crowning, cord, breech/limb presentation, or meconium.
4. Delivery imminent?	Crowning, uncontrollable urge to push, frequent contractions, advanced parity, or unsafe transport: prepare for field delivery.
5. Normal or complicated?	Normal delivery, prolapsed cord, breech, shoulder dystocia, or multiple gestation. Request additional resources early when needed.
6. Mother + newborn	Control bleeding, maintain warmth, record delivery and APGAR times, use AO-02 for newborn care, and notify destination early.

Key treatment and disposition

Situation	Approved action
Normal delivery	Support the head and body without pulling. Check for nuchal cord. Dry and warm the infant. Do not pull on the infant, cord, or placenta.
Prolapsed cord	Hips/knees to chest. Use a gloved hand to relieve pressure from the presenting part. Cover exposed cord with a moist saline dressing and transport emergently.
Breech	Support presenting parts without pulling. If the head does not deliver, create an airway space at the infant's nose. Do not pull on the infant.
Postpartum hemorrhage	After placental delivery, perform uterine massage. For hemorrhage with shock within 3 hours of delivery, give tranexamic acid 1 g IV/IO over 10 minutes and use AM-05.

Oxytocin is not carried on the Claiborne formulary and is not authorized under AO-01.

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PROVIDER ACTIONS, SAFETY, AND RELEASE CONTROL

Provider-level actions

Level	Actions
EMT	Left lateral positioning when delivery is not imminent; document contraction frequency/duration; support uncomplicated delivery and listed complications; warm newborn; request ALS/resources early.
AEMT	All EMT actions plus IV/IO access and protocol-directed treatment of maternal hypotension or hemorrhage without delaying delivery or transport.
Paramedic	All preceding care plus advanced management of maternal shock, seizure, airway compromise, complicated delivery, and approved TXA for qualifying postpartum hemorrhage.

Safety checks

- Do not perform a digital vaginal examination.

- Delay cord clamping for at least 60 seconds in a preterm infant under 37 weeks who does not require immediate resuscitation.

- Uterine massage is performed only after delivery of the placenta.

- Large-volume or free vaginal bleeding is abnormal. Treat shock immediately and minimize transport delay.

Linked pathways

Pathway	Use when
AO-02 Newly Born	Immediate newborn assessment and resuscitation
AO-03 OB-GYN Emergency	Hemorrhage, seizure, pregnancy emergency, or eclampsia
AM-05 Hypotension / Shock	Maternal shock or inadequate perfusion

Source and release control

- Claiborne County EMS approved childbirth/labor and postpartum hemorrhage content.

- Claiborne County EMS medication formulary and linked protocol pathways.

- This is the controlled Claiborne source PDF and matches the approved Claiborne EMS app content.

- Document focused findings, interventions with dose/route/time/response, serial reassessments, destination decision, and notifications.